

Discharge Summary

Internal Medicine — Admission 22 Sep 2026 to Discharge 23 Sep 2026

Patient

Patient name	Layla Ahmed
MRN	MRN-90344
Date of birth	29 Jul 2001
Admission date	22 Sep 2026
Discharge date	23 Sep 2026
Attending physician	Dr. Faisal S., Outpatient Internal Medicine

Reason for Admission

Ms. Ahmed was admitted for a short observational stay following an episode of acute epigastric pain associated with vomiting, to rule out an acute surgical abdomen prior to outpatient gastroenterology follow-up.

Hospital Course

On admission, vital signs were stable (BP 110/70, HR 72, afebrile). Abdominal examination revealed mild epigastric tenderness without rebound or guarding. Bedside ultrasound of the abdomen was unremarkable, with no evidence of gallstones, appendicitis, or free fluid. Basic metabolic panel and lipase were within normal limits, effectively ruling out pancreatitis. The patient was managed conservatively with IV fluids and antiemetics, with symptomatic improvement over 18 hours.

Hospital Course (continued)

By the morning of the second hospital day, the patient was tolerating oral intake without recurrence of vomiting, and pain had reduced from 7/10 to 2/10 on a verbal analog scale. Given the unremarkable workup and clinical improvement, the differential was narrowed to functional dyspepsia, pending the gastroenterology referral already placed on 24 Sep 2026 for further outpatient evaluation.

Discharge Medications

Omeprazole 20mg once daily before breakfast, for 4 weeks. No other new medications prescribed at discharge. Patient was reminded of her documented sulfa drug and latex allergies, which were confirmed again prior to discharge and clearly flagged in her chart.

Follow-up Instructions

1. Gastroenterology follow-up with Dr. K. Farooq as per the referral letter dated 24 Sep 2026.
2. Return to the emergency department immediately if pain worsens, if vomiting recurs, or if there is any sign of gastrointestinal bleeding (black stools, blood in vomit).
3. Maintain a bland diet for the next week, avoiding spicy and fatty foods.
4. Routine outpatient review with Dr. Faisal S. in 2 weeks if gastroenterology evaluation is still pending.

Condition at Discharge

Stable, ambulatory, tolerating oral intake, pain well controlled. Discharged home in the care of a family member.